

EPILEPSY: ROUTINE CARE

- If fitting now →19. If not known with epilepsy and has had a recent fit →19 to assess further.
- A doctor must confirm the diagnosis of epilepsy and start long term anticonvulsant medication.

Assess the patient with epilepsy

Assess	When to assess	Note
Symptoms	Every visit	Ask about fit frequency and review fit diary. Manage other symptoms as on symptom pages.
Adherence	Every visit	Ask if takes treatment every day. If not, explore reasons, support adherence and refer to community health worker.
Side effects	Every visit	Ask about side effects of treatment ↪ 150. If side effects intolerable, switch anticonvulsant.
Other medication	Every visit	If patient on any other medication (especially TB treatment, ART or contraceptive), consider possible interactions: check SAMF or discuss with MIC hotline ↪ 178.
Family planning	Every visit	• Assess patient's contraceptive needs ↪ 154. • If woman of child-bearing potential on sodium valproate, discuss the risk of birth abnormalities² and advise to switch anticonvulsant. If patient agrees, switch to lamotrigine ↪ 150. If patient wishes to continue valproate, ensure patient on reliable contraception³ and have patient sign risk acknowledgment form⁴ yearly. • If pregnant or planning pregnancy: discuss/refer to specialist. Give routine antenatal care ↪ 159 and give folic acid 5mg daily. Refer to high risk antenatal clinic within 2 weeks.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↪ 143.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↪ 142.

Advise the patient with epilepsy

Health for All

↪ 124

- If newly diagnosed, refer to community health worker and Epilepsy South Africa for support ↪ 178. Help to get a MedicAlert® bracelet ↪ 178.
- Advise to keep a fit diary to record frequency and duration of fits, triggers and changes in medication. Educate about the need for adherence and to continue treatment even if no fits.
- Help identify and avoid triggers like lack of sleep, alcohol/drug use, dehydration, flashing lights and video games.
- Help reduce chance of injury: advise to avoid dangers like heights, fires, swimming alone, walking/cycling on busy roads, operating machinery. Advise to avoid driving until fit free for 1 year.
- Advise patient there are many medications that may interact with anticonvulsants (see table ↪ 150) and to discuss with doctor before starting any new medication.

Treat the patient with epilepsy

- **If not on treatment:**
 - Choose an anticonvulsant based on if patient is a man or woman, child-bearing potential and other medication ↪ 150.
 - Start a single anticonvulsant at low dose and increase until fits stop or side effects intolerable.
- **If already on treatment:**
 - If woman of child-bearing potential on sodium valproate, discuss risks² and explain the need to switch anticonvulsant.
 - If no further fits, continue same dose.
 - If still having fits:
 - If poor adherence: support adherence, continue same dose and review patient in 2 weeks.
 - If medication interactions: adjust medications as needed and review patient in 2 weeks.
 - If none of above: increase anticonvulsant dose ↪ 150. If already on maximum dose for 4 weeks, switch anticonvulsant once ↪ 150. If already on second anticonvulsant, avoid switching and refer instead.
- **If switching medication:** add new anticonvulsant and increase as needed. Continue old anticonvulsant for first 2 weeks, then slowly reduce dose over 6-8 weeks, until old anticonvulsant stopped.

Continue to treat the patient with epilepsy →150.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²If woman on sodium valproate becomes pregnant, risks to baby include problems with development of spine, brain and other learning problems. ³Reliable contraception includes intrauterine device (IUD), subdermal implant, injectable or sterilisation. ⁴Form available from: www.sahpra.org.za/wp-content/uploads/2022/08/GLF-CEM-PV-S01_v1-Valproate-Annual-Risk-Acknowledgement-Form.pdf